TEAM REFLECTION

Use Case:

Copay / Patient Support Program Analytics

13/08/26

Day 1 : Understanding the Use Case:

Initial Understanding

- Our team received the use case under the **Patient Services** domain.
- Initially, the team was unclear about:
    - What a **copay program** is.
    - What a **Patient Support Program (PSP)** is.
    - Who owns/runs these programs.
    - Who uses the programs.
    - How patient and pharmacy data reaches pharmaceutical companies.
    - Whether patients can choose between multiple copay programs.
    - What exactly the expected product/system should be.
    - How the use case can provide value to patients.

1. Today was mainly about understanding what our use case actually means before deciding what to build.
2. When we first saw “Copay / Patient Support Program Analytics”, we were honestly quite confused about what a copay program was and what exactly was expected from us.
3. One of our first questions was who actually runs these programs and why a pharmaceutical company would spend money on them.
4. We understood that a copay program can help reduce the amount a patient has to pay for a medicine, making it easier for eligible patients to afford and continue their treatment.
5. We also clarified that a Patient Support Program is broader than just copay assistance. It can include things like financial assistance, insurance/access support, enrollment help, refill support, and other services depending on the program.
6. We initially thought that the patient might be able to compare multiple copay programs and choose whichever one gives them the most

benefit. After discussing the use case, we realized that this is not what our given problem is primarily asking us to build.

7. Another major question we had was how pharmaceutical companies get access to patient-related information, considering privacy regulations. We discussed that the data can come through authorized parties and healthcare data partners, and that our project should work with de-identified/synthetic data rather than real patient-identifiable information.
8. We then started understanding why the problem statement specifically mentions “copay program transaction data” and “pharmacy claims.”
9. We understood that copay transaction data can tell us about how the support program was used and how much assistance was provided, while pharmacy claims can help us understand whether patients continued filling their medication.
10. The biggest clarification for us was understanding the phrase “cohort comparison (enrolled vs. not)”. Basically, we need to compare patients who used/enrolled in the program with a suitable comparison group that did not, and see whether there is a difference in their treatment continuation.
11. This led us to understand the importance of adherence and persistency. In simple terms, we are trying to understand whether patients are continuing their medication and for how long.
12. We also realized that the project isn't only about asking “Does the program work?”. We also need to ask “Is the money being spent on the program worth the outcome we are getting?”
13. This is where the cost-per-adherent-patient metric becomes important. We need to connect the program spending with the number of patients who achieve the desired treatment outcome.
14. From the patient's perspective, the program is meant to make access to treatment easier, while from the pharmaceutical company's perspective, the analytics helps them understand whether their support program is actually helping patients and whether the program is cost-effective.
15. At this stage, we feel that our main product is likely to be a program-effectiveness analytics dashboard for the pharmaceutical/Patient Services team, rather than a patient-facing app.
16. We still have several things to figure out, especially what exact data we will use, how we will calculate adherence and persistency, how we will compare the two cohorts fairly, and whether we can meaningfully calculate ROI with the data available to us.

17. Overall, today's discussion was less about building and more about understanding the business problem properly first. We started with very basic questions about what a copay program even is, and by the end we had a much clearer idea that the core problem is about connecting patient support spending with patient treatment outcomes and program effectiveness.

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Day 2 : Review 1:

Summary

In this meet, we explained our understanding of the use case and we also presented our ideas and approach. Our mentor gave some insights and suggestions, they were:

• **Beyond a basic dashboard:** The project should show deeper analysis, including the mathematics behind the effectiveness metrics, how they are calculated, and what the results actually mean for the business.

• **Bring in Agentic AI:** Find practical places where an agentic AI workflow can add value rather than using AI just for the sake of it. We should identify specific cases where an agent can analyze results, identify issues, and suggest actions.

• **Explore social-media data:** Investigate whether social-media data can provide useful insights such as patient sentiment or feedback, and assess whether it is realistically feasible to collect, process, and integrate within the hackathon timeline.

• **Strengthen the business perspective:** The main goal should be to find the balance between business efficiency and patient benefit. Recommendations should help pharma optimize spending without compromising patient access or outcomes.

• **Demonstrate real data processing:** Instead of relying only on ready-made CSV files, Try to generate realistic non-CSV datasets and demonstrate how our system processes and transforms the raw data into meaningful analytics.

• **Show the analysis, not just the output:** Include the calculations, statistical reasoning, visualizations/plots, and potentially the underlying code so that the technical depth of the solution is clearly demonstrated.

• **Maintain a GitHub repository:** Keep the project organized in a shared GitHub repository so the team can collaborate effectively and the mentor/judges can see the actual implementation and technical work.